SAMPLE ANSWER · CEREBRAL PALSY · CASE 1

Sample 7-minute ISBAR: Amira

Amira Hassan · 4-year-old girl · Paediatric community occupational therapy home visit · about 1043 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

62 words

"Hi, I'm the occupational therapy student who has been completing a home seating review for Amira Hassan. Amira is a 4-year-old girl with spastic quadriplegic cerebral palsy, GMFCS level V. I need to discuss her with you urgently because I'm concerned she may have an injury to her left leg that needs same-day medical review, and I have paused the seating trial."

S Situation

121 words

"When Amira's mum, Leila, straightened her left leg for a nappy change, Amira screamed and cried for several minutes. Her left thigh looks fuller than the right and feels warmer, she cries when her left knee is bent, and she's holding that leg stiffly. Leila says Amira has been cranky for two days and sleeping poorly, and she has been giving Panadol since yesterday because she thinks Amira is teething. Leila says there has been no fall, and I can't see any bruising.

"Meanwhile, the supplier needs measurements within ten minutes, Leila has to leave at 2.45 to collect Amira's brother from school, and she's worried about the NDIS plan review on Friday. I don't think we should continue the trial."

B Background

160 words

"Amira lives in Logan with her parents, Leila and Omar, and her 8-year-old brother, Yusuf. She is GMFCS level V and MACS level V, communicates without words through facial expression, sounds and crying, and is fully gastrostomy fed. She takes regular medication for tone and seizures; I'd need to confirm which medications, and exactly how much paracetamol she's had and when.

"Reassuringly, her hip surveillance X-ray in June showed stable hips, the dietitian documented steady weight gain last month, and Leila reports no recent seizures.

"There are important gaps. There's no documentation about Amira's bone health or previous injuries, which carers have handled her this week, or when she last saw her paediatrician. Leila mentioned that Amira cried throughout a bath with a support worker earlier this week, which is unusual, but couldn't recall the day. Omar spoke to Leila in Arabic, I'm not sure how much he understood, and the referral doesn't say whether either parent needs an interpreter."

A Assessment

321 words

"My main concern is that the cluster of signs — pain when the left leg is moved, a swollen, warm thigh, guarding, and two days of irritability and poor sleep — is localised to one limb and isn't what I'd expect from teething. It could indicate a fracture or another acute problem in the leg, such as an infection, and it needs same-day medical assessment.

"In cerebral palsy, children who are non-ambulant at GMFCS level V often have low bone density because of limited weight bearing, and nutrition and some anticonvulsant medications can add to that risk. So a fracture can happen with very little force, during a routine nappy

change, bath or transfer, without anyone noticing a specific event. Because Amira can't tell us where it hurts, crying with handling and irritability may be our only signs, and the paracetamol may be masking some of her pain.

"Although her hips were stable in June, that X-ray tells us nothing about her thigh today.

"Occupationally, this affects every activity that involves handling her: nappy changes, bathing, positioning, sleep and feeding. Continuing the seating trial could cause more pain and possibly further injury, so the wheelchair, though important, has to wait.

"There's also a safeguarding consideration. Any unexplained injury in a young child needs to be documented carefully and considered under our child protection procedure. That isn't the same as suspecting the family. Leila has been open with me, and there's a known medical reason Amira could fracture easily. It's not my role to decide what happened; my role is to record what I observed objectively and escalate it with you.

"Both parents also need to understand what's happening. I shouldn't rely on Leila to interpret clinical information for Omar, especially about a possible injury.

"So my priorities are: first, gentle handling and same-day medical review; second, escalation and our safeguarding procedure; third, clear communication with both parents; and the wheelchair afterwards."

R Recommendation

379 words

"My immediate recommendation is that we stop the seating trial now. I'd help Leila move Amira out of the trial chair onto the mat, supporting her left leg in whatever position is most comfortable and avoiding straightening or rotating it. I'd let the supplier know politely that we're postponing.

"Second, I'd like your help to arrange same-day medical review. Depending on our service protocol, that might be her GP or paediatrician today, or the emergency department, so she can be examined and imaged. We'd advise the family how to transport her with the leg supported, and help them plan Yusuf's pick-up. I'd also give medical staff the paracetamol timing and doses.

"Third, I'll document exactly what I saw and heard, with Leila's words and times, and I'd like us to work through the safeguarding procedure together today. You may need to consult the service's child protection lead or consider a report under the Child Protection Act 1999. I'd explain to Leila honestly that this is a standard step whenever a child has an unexplained injury, so she doesn't feel blamed.

"Fourth, I'd ask whether Omar would like a professional interpreter, through TIS National, for the follow-up call and medical handover, so both parents can ask questions.

"For the team, I'd ask the treating doctors to assess the leg and, if a fracture is confirmed, to review her bone health. I'd ask her physiotherapist to help plan safe handling and positioning while she heals, the dietitian to review nutrition for bone health, and social work to support the family with stress and sibling care.

"For OT, once Amira has been medically reviewed, my principal short-term assessment would be an observed handling and positioning assessment with Leila and the support worker, watching nappy changes, bathing and transfers, so we can teach safe techniques and adjust

equipment. Longer term, I'd use the COPM with Leila and Omar to capture their priorities, likely including the wheelchair, communication access and toileting.

"For the NDIS deadline, I'd contact the support coordinator to explain that the seating assessment was postponed for medical reasons and document this, so the family isn't disadvantaged. I'd pass the communication question to speech pathology.

"I'd follow up with the family tomorrow, then rebook the seating review once Amira has been cleared."

Coaching notes *not spoken*

The immediate priority

Amira has localised pain, swelling and warmth in the left thigh with two days of irritability and no reported injury. In a non-ambulant child at GMFCS V this pattern needs same-day medical review, gentle handling and a stop to the seating trial, with the unexplained injury handled through safeguarding procedure without blaming the family.

Distractors in this case

- **NDIS plan review deadline and wheelchair quote** — real pressures, but a short delay with documented reasons is far safer than measuring a child who may have a fracture.
- **Teething and paracetamol** — a plausible family explanation, but it does not account for pain on moving one leg with a warm, swollen thigh; paracetamol may also mask pain.
- **Stable hip X-ray and good weight gain** — reassuring about other issues, but they say nothing about the femur today.
- **AAC and toilet-training questions, sibling pick-up** — important to the family; acknowledge them and schedule them, rather than letting them drive the visit.

Missing information to clarify

- Medications, doses, and exact paracetamol timing
- Bone health history, previous injuries, last paediatric review
- Who has handled Amira this week, including the support worker's bath
- Interpreter preference for each parent

Useful references

- Child Protection Act 1999 (Qld) and your service's child safety procedure
- TIS National (Translating and Interpreting Service)

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